Question:

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?Answer Choices:A. Neonatal respiratory distress syndrome (NRDS)B. Tracheoesophageal fistulaC. PneumoniaD. Congenital heart anomaly with right-to-left shunt

Reasoning + Answer:

<think> The child was born after the 32nd gestational week, which rules out severe congenital heart anomalies with significant cyanosis. The child has signs of respiratory distress such as tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. These symptoms are more consistent with a respiratory issue. The presence of subcostal and intercostal retractions and nasal flaring are characteristic of respiratory distress syndrome, especially in premature infants, but the child is post-term. Neonatal respiratory distress syndrome (NRDS) typically affects preterm infants. The child's respiratory distress and cyanosis are likely due to an obstruction or a condition affecting the airways, rather than a congenital heart anomaly. The signs of respiratory distress and the response to oxygen administration suggest an obstruction, such as a tracheoesophageal fistula, which can cause aspiration and subsequent respiratory distress. Pneumonia can cause similar symptoms but would not respond as well to oxygen administration. The signs of obstruction and the response to oxygen administration are consistent with a tracheoesophageal fistula. </think><answer>B. Tracheoesophageal fistula</answer>